



Medication: Zuclopenthixol Acetate (Clopixol Acuphase®) (Mental Health) Policy

ALERT • **Do not** confuse with Zuclopenthixol Decanoate (Clopixol Depot®) which is a long-acting intramuscular (IM) injection used as long term maintenance treatment in chronic psychoses.

Preamble

Consumer Participation

The Rockingham Peel Group (RKPG) recognises and fully supports the need for the patient and/or their primary carer to¹:

- Know and exercise their healthcare rights.
- Be engaged in their healthcare.
- Have access to information about treatment options.
- Participate in treatment decisions.
- Have access to information about agreed treatment plans.

Scope of Practice

It is the responsibility of each health care provider to ensure all patient care and therapeutic interventions they provide are within their individual scope of practice. Refer to the [RKPG Guidelines to Writing for Clinical Policy](#) for further information about Scope of Practice.

Indications

- Acute psychosis and acute mania where other treatment modalities have been ineffective.³⁻⁵
- Zuclopenthixol Acetate should be reserved for those patients who:
 - * Have required repeated IM medication.
 - * Have had sufficient time for assessment of response and/or side effects to previously injected drugs (minimum of 60 mins for IM and 15 mins for IV)

or

- * Have previously received Zuclopenthixol Acetate and have shown good tolerability and response to it.^{3,5}

Precautions

Zuclopenthixol Acetate should be used with caution in patients who:

- Are receiving other antipsychotics - may lead to over-sedation which is difficult to reverse.
- Are sensitive to extrapyramidal side effects (EPSEs).
- Have pre-existing cardiac disease, hepatic or renal impairment.
- May be pregnant.

RESPONSIBILITY				NATIONAL STANDARDS
First Issued	Feb 2018	Revisions	NA	National Standards 1, 4, 5, 6
This version	Apr 2021	Revision Due	Aug 2025	
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- Are unable to/do not consent to injection – **take care if patient is struggling** it must be given IM (dangerous if accidentally given into a vein).^{3,4}

Classification

Thioxanthene neuroleptic, intermediate-acting antipsychotic injection.⁴

Presentation

50mg/1mL or 100mg/2mL glass ampoules; (protect ampoule from light. Store below 25°C).⁴

Consent

Wherever possible, aim for patient consent for treatment unless under the Mental Health Act 2014. If under the Mental Health Act information must be provided to the client as soon as possible.

Dosage and administration

- Note:** Consultant approval is required and must be documented in medical record.
- Usual dose is 100mg IM stat (licensed dose is 50-150mg) inject slowly into a large muscle.²⁻⁵
- There is no evidence to support higher doses over lower doses.^{2,3}
- Females, neuroleptic-naïve patients and the elderly may require lower doses of 25-50mg.
- Elderly patients should not receive more than 100mg.
- The dose may be repeated if necessary after 48 - 72 hours.³⁻⁵
- Maximum dose is 400mg or 4 doses (whichever comes first) over 2 weeks.³⁻⁵
- Patients must be assessed prior to each dose - Zuclopenthixol Acetate should **not** be prescribed as a “course of treatment”).³
- Zuclopenthixol Acetate should be prescribed in the ‘once only’ section of the WA HMC (RGMR170) and care taken to ensure the Zuclopenthixol Acetate IM injection is prescribed, as there are other formulations of Zuclopenthixol with very different properties.
- All other parenteral antipsychotics should be withheld when patients are receiving Zuclopenthixol Acetate, including medications with a PRN dose frequency within a 24hr time period post administration of IM Zuclopenthixol Acetate.³
- Zuclopenthixol Acetate may need to be given with IM lorazepam or midazolam if immediate sedation is required. (**Note:** must be given separately; cannot be mixed in the same syringe).⁵
- Benztropine 1-2mg oral or IM injection up to **three** times daily when required should be prescribed on the ‘PRN AS REQUIRED MEDICATIONS’ section of the WA HMC (RGMR170).^{4,5}
- If initiating a Zuclopenthixol Decanoate depot, it is possible to administer it in the same syringe with the last injection of Zuclopenthixol Acetate.⁴

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Adverse effects⁴

- Hypersensitivity
- Sedation
- Extrapyrimal side effects
- Orthostatic hypotension
- Tachycardia

Monitoring

Sedation

- Zuclopenthixol Acetate **should not** be used for rapid tranquilisation as the onset of action occurs at 2 hours and peaks at 24-36 hours.
- The first injection is usually the most sedating. Sedation may initially be seen within 2 hours after injection, peaks after 12 to 36 hours and may last up to 3 days.^{4,5}

Extrapyrimal side effects

- Review carefully for ESPEs and treat rigorously if they occur.
- Benztropine 1-2mg oral or IM injection up to **three** times daily when required (up to 6mg/24hours) can be administered from the 'PRN AS REQUIRED MEDICATIONS' section of the WA HMC (RGMR170). Anticholinergic drugs should not be given routinely for prophylaxis particularly in older patients.^{4,5}

Physical signs and symptoms

- ECG prior to administration if possible. Document if unable to perform ECG
- Patients must be carefully monitored following each injection. Physiological observations must be recorded on the Adult Observation and Response Chart (RGMR141.A)
- Blood pressure (BP), temperature, oxygen saturations, heart rate (HR) and respiratory rate (RR) must be monitored every 15 minutes for the first hour, then at 2,3,4,6 and 8 hours post-injection and then every 4 hours until 36 hours has elapsed.
- If the patient is uncooperative but fully alert and ambulating, physiological observations are not required but it **must** be documented that the patient was visually assessed and a description that they were alert and ambulating.

Compliance monitoring

- Medication Datix incidents will be reviewed.

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References

1. Australian Commission on Safety and Quality in Health Care (ACSQH). The National Safety and Quality Health Service Standards Safety and Quality Improvement Guide: Standard 1 Governance for Safety and Quality in Health Service Organisations. 2nd ed. 2017. Available from <https://www.safetyandquality.gov.au/our-work/assessment-to-the-nsqhs-standards/>
2. Gibson R, Gunadasa S, Jayakody K, Kumar A. Zuclopenthixol acetate for acute schizophrenia and similar mental illnesses. 2012. Cochrane Database Systematic Review. Available from http://www.cochrane.org/CD000525/SCHIZ_zuclopenthixol-acetate-for-acute-schizophrenia-and-similar-serious-mental-illnesses
3. Kapur S, Paton C, Taylor D. The Maudsley Prescribing Guidelines in Psychiatry (12th Ed). 2012. New Delhi: Wiley-Blackwell.
4. MIMS Online. Clopixol Product Information. 2017. Available from https://www.mimsonline-com-au.smhslibresources.health.wa.gov.au/Search/FullPI.aspx?ModuleName=ProductInfo&searchKeyword=clopixol&PreviousPage=~/Search/QuickSearch.aspx&SearchType=&ID=32810001_2
5. Therapeutic Guidelines Pty Ltd. Psychotropic Guidelines. 2017. Available from <https://tgldcdp-tg-org-au.smhslibresources.health.wa.gov.au/topicTeaser?guidelinePage=Psychotropic&etgAccess=true>

Links

[ACSQH: Medication Safety Standard 4.1](#)

[DoH High Risk Medication Policy](#)

[RKPG Medication Administration Clinical Practice Standard](#)

Acknowledgement

We acknowledge the following previous site endorsed work and/or contributors used to compile this policy:

- Fiona Stanley and Fremantle Hospital and Health Service, Guidelines for the use of Zuclopenthixol Acetate® (Acuphase®) issued Jan 2015 Liana Shymko - Senior Pharmacist, Alma Street Centre FHHS
- North Metropolitan Health Service, Mental Health. Area Mental Health Policy Manual – Zuclopenthixol Acetate (Clopixol Acuphase®): Prescribing Guidelines Issued 22 May 2013. NMHS Mental Health Executive Group

National Standards

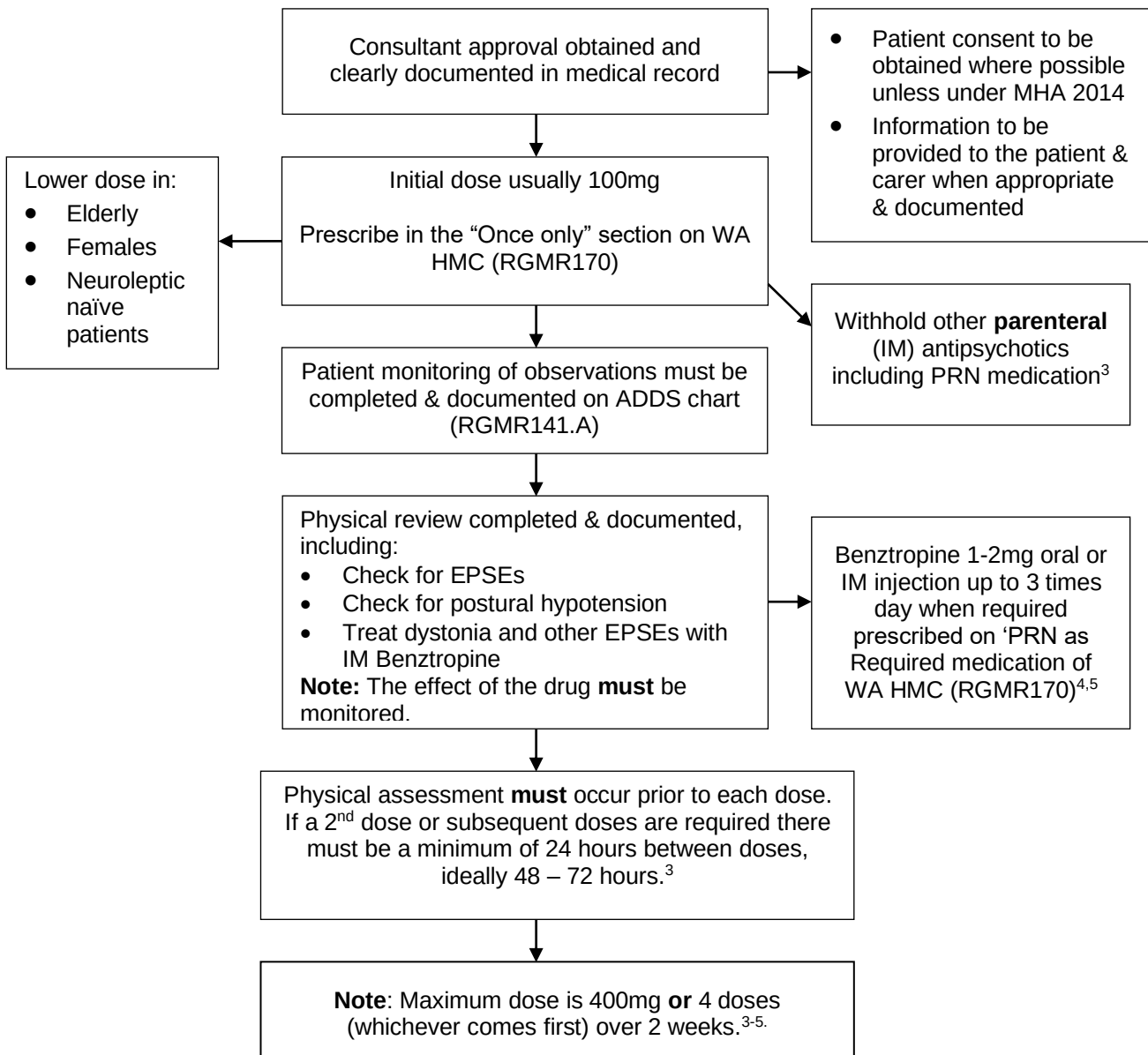


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Appendix A: Flowchart for Use of Zuclopenthixol Acetate

Criteria met for Zuclopenthixol Acetate (Clopixol Acuphase®):

- Acute psychosis and acute mania where other treatment modalities have been ineffective
 - Not first line treatment; patient has required repeated IM medication
 - Have had sufficient time for assessment of response and/or side effects to previously injected drugs (minimum 60 mins for IM and 15 minutes for IV)
- OR**
- Have previously received Zuclopenthixol Acetate and have shown good tolerability and response to it



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